



MY DOCTOR
FAMILY PRACTICE

EDITED: August 2026

GENDER-AFFIRMING HEALTHCARE GUIDELINE

Client information for feminising hormone therapy

Important disclaimer: This guide is a practical resource developed by My Doctor Family Practice. It summarises selected recommendations from established gender-affirming healthcare guidelines, including the Southern African HIV Clinicians Society (SAHCS) 2021 guideline, and incorporates additional practice-specific clinical and administrative information. It is not an official SAHCS guideline, does not replace the original guidelines or individual clinical assessment, and should not be used to independently start, stop or change treatment. Clinical decisions must be individualised and based on current evidence, applicable guidelines and the patient's circumstances.

Introduction

Many transgender and gender diverse (TGD) clients choose to seek gender-affirming care in the form of hormone therapy. The decision to commence hormone therapy rests with you, the client, and not with your healthcare provider. The informed consent model of treatment gives you agency over this decision – it is the role of your doctor to support and guide you through this process safely and effectively.

You may have read up, or heard from other TGD persons, about hormone therapy. Some of the information that you may have come across could potentially be out of date or inaccurate.

If you have any questions or concerns at any time, then you should always feel free to raise these concerns with your healthcare provider.

Please remember that every client is unique and will respond differently to medication, and that one client's treatment programme might differ substantially from another's due to a variety of physiological and medical factors. Try not to compare the treatment that you are receiving with that of anyone else – your journey is your own. If you have concerns about the efficacy of your treatment, then make a point to discuss this with your provider at your next appointment.

Before commencing hormone therapy, there are a few points that are worth considering. This information document will draw your attention to some of these points, as well as

outlining the expected effects, side effects and risks associated with hormone therapy, to ensure that you have all the information you need to make the best decision about your body and your health.

The role of endocrinologists and other specialists

Hormone therapy does not need to be prescribed or monitored by an endocrinologist. Prescribing hormone therapy is well within the realm of a suitably skilled general practitioner (GP) or family physician. Not all GPs, nor all endocrinologists, have experience in managing gender-affirming hormone therapy, and the expertise of the clinician should be the guiding factor in determining who prescribes and monitors your hormone therapy.

You may benefit from seeing other allied health professionals, such as speech-language therapists, or from medical specialists such as plastic surgeons. Not every TGD client will necessarily want to pursue these options, and you should discuss your individual goals with your prescribing doctor.

The role of psychotherapy in TGD clients

Seeing a psychologist is not a requirement for initiating hormone therapy. While previously, some healthcare providers required a letter of diagnosis or referral, this is no longer necessary under international best practice.

Your doctor will ask you some questions to determine that you have a good support structure as you move forward. This is not because taking hormone therapy itself necessitates this; rather, as many TGD clients experience, navigating the world is already difficult, and hormone therapy – although often resulting in many positive and beneficial changes – can also lead to some emotional lability. A solid support structure looks different for everyone, but may include friends, family, support groups, therapists and counsellors. Your doctor may suggest or offer you referrals to support groups and therapists, if you indicate that you may benefit from these.

Other important aspects to explore:

Potential impact on emotions: The impact of hormones can be very diverse and individualised. Your mood may fluctuate. For example, some TGD women may experience being more moody or tearful at times.

Potential impact on relationships with family and significant others: Have you thought about the possible impact on your relationship with significant others? If you are in an intimate relationship, this may change when you start on hormones, and relationship roles may need to be re-negotiated. A partner may grieve the loss of aspects of who you were and the way the relationship used to be. What possible impact can it have on your family and how will you be able to deal with your family's response? Have you considered the impact of the change of gender role in your family? Have you

considered the impact of potential loss of fertility? Are there children that may be impacted and are the children prepared?

Potential change in sexual orientation: It is possible that your sexual orientation may remain constant or shift, either temporarily or permanently (e.g. shift in attraction or choice of sexual partners, widened spectrum of attraction, shift in sexual orientation identity)

Potential impact on safety: In some settings, the physical changes on hormones may affect your safety, with persons who do not fit into society's expectations of male or female being at increased risk of violence.

Potential impact on employment: Some TGD persons experience discrimination in the workplace, or struggle to obtain employment. This may be more difficult when your legal documents (e.g. ID) and your appearance do not match.

Potential grief and loss: Some TGD persons experience a sense of loss. A TGD man may lose certain gender roles in the family.

Taste changes: Some TGD persons experience a change in their taste sensations, and their likes and dislikes of certain foods.

Body odour change: Some TGD persons experience a change in their body odour on hormone therapy.

Appetite and sleeping patterns: Often the TGD person will experience an increase in appetite. This could cause weight gain. Sleeping patterns may also be affected.

Feminising hormone therapy

Feminising hormone therapy is prescribed for assigned male-at-birth clients who wish to feminise. The backbone of feminising hormone therapy is oestrogen therapy. Anti-androgens (ie. spironolactone) are sometimes used, although many clients can achieve testosterone suppression using oestrogen therapy alone.

The biggest concern with oestrogen therapy is the risk of clot formation, which can lead to deep venous thrombosis (DVT) or life-threatening pulmonary emboli. It is this risk that limits the amount of oestrogen that healthcare practitioners can safely prescribe to clients. Oral oestrogen (Estrofem or Premarin, for example) carries a higher risk for these adverse events than injectable or topical oestrogen preparations.

Your doctor will discuss with you the various options for hormone therapy that are available, and help you to decide which form of treatment is best for you.

Comparison of oestrogen preparations			
Type	Examples	Advantages	Disadvantages
Tablet, taken orally	Estrofem Premarin	• Easy to use • Accessible at most pharmacies	• Higher thrombotic risk
Tablet, dissolved sublingually†	Estrofem	• Accessible at most pharmacies	• Time consuming (takes 30 minutes to absorb)
Injectable	Oestradiol valerate	• Safer • Affordable • Once-a-week dosing	• Available only from compounding pharmacy • Requires knowledge of injection technique • Requires disposables (syringes, needles, alcohol swabs)
Patch	Estradot	• Safer • Available at most pharmacies • Twice-a-week dosing	• Patches need to be applied and cared for correctly • May cause skin reactions due to adhesive

†, Taking an oestrogen tablet sublingually involves holding the tablet in the mouth – either under the tongue, inside the cheek, or between the lips and teeth, while the tablet dissolves. After dissolving, the residue must be held in the mouth for a further 30 minutes to fully absorb, and any remnants need to then be spat out and the mouth thoroughly rinsed. It is important not to swallow at all during this time, as any swallowed medication will go through the digestive tract and carry a higher risk of adverse events.

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Additional medications:

When taking **oral feminising hormone therapy**, oestrogen alone is often not enough to lower testosterone levels to the desired range. For this reason, an additional medication called a testosterone blocker (anti-androgen) is usually prescribed. These medicines help reduce the effects of testosterone, allowing the oestrogen to work more effectively in supporting feminising changes. Your doctor will monitor your hormone levels and adjust treatment as needed to ensure it is both safe and effective.

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Changes that occur when using hormone therapy

The changes you will experience on hormone therapy often take some time to develop fully. Some of these changes are reversible and will disappear should you discontinue hormone therapy. Others are irreversible and will persist even if you stop taking your hormones.

The timeline for these changes to begin is variable, but most of them will only reach their maximum degree after 3–5 years on hormone therapy.

Reversible changes

Such changes include:

- loss of muscle mass and decreased strength
- changes in body fat distribution, possibly associated with weight gain (increased fat deposition in breasts, buttocks, hips, and thighs)
- softer and thinner skin
- reduced acne
- lighter and thinner body and facial hair
- cessation of male-pattern balding, possible scalp hair regrowth
- changes in sex drive (usually a decrease initially, followed by an increase together with a change in sexual response cycle)
- changes in strength and frequency of erections, and changes in the amount and consistency of ejaculate
- changes in mood and emotional response.

Irreversible changes

Such changes include:

- breast development; while the size of breast tissue may fluctuate, hormone therapy will cause permanent development of breast structures that will remain even if hormone therapy is withdrawn
- testicular atrophy
- infertility
- changes in bone density.

TABLE 7: Timeline and reversibility of feminising hormone therapy.^[260]

Effect	Time from initiation to onset	Time from initiation to maximum effect	Reversible
Body fat redistribution	3–6 months	2–3 years	Yes
Decreased muscle mass and strength	3–6 months	1–2 years	Yes
Skin softening	3–6 months	Unknown/variable	Yes
Change in sexual desire	1–3 months	3–6 months	Yes
Decreased erections	1–3 months	3–6 months	Yes
Breast growth	3–6 months	4 years	No
Decreased sperm production	Unknown/variable	>3 years	Possibly
Decreased terminal hair growth	6–12 months	>3 years	Yes
Scalp hair	Variable	Unknown/variable	Yes
Voice change	None	n/a	n/a

Tangpricha V, den Heijer M. Oestrogen and anti-androgen therapy for transgender women. *Lancet Diabetes Endocrinol.* 2017;5(4):291–300.

Limitations of hormone therapy

It is important to understand that there are certain features that hormone therapy cannot alter. Among others, these include:

- presence of facial hair – although hormone therapy may make the hair thinner, or cause it to grow more slowly, hormone therapy will not eliminate facial hair
- pitch of the voice
- bone structure of the face
- presence of thyroid cartilage (Adam’s apple).

Important risks associated with hormone therapy

As with any medication, hormone therapy carries with it certain risks. Some of these risks can be mitigated or reduced by lifestyle factors, while others are independent risks that cannot be altered. It is important to be fully aware of the risks associated with hormone therapy before making the decision to start your treatment.

Blood clots are the most prominent risk associated with feminising hormone therapy. A blood clot can lead to deep venous thrombosis (DVT), pulmonary embolism (a blood clot in the lungs), heart attack or strokes. These conditions may be severely debilitating or even fatal:

- cardiovascular disease
- nausea or vomiting
- migraines or other headaches
- gallstones and other diseases of the gallbladder
- elevated levels of prolactin can rarely occur in clients on feminising hormone therapy, due to the development of a prolactinoma, a benign (noncancerous) tumour of the

pituitary gland, that may interfere with vision. These can require surgical management, depending on the nature of the lesion.

Some of the risks mentioned are modified by other factors. Notably, cardiovascular and clot risk are worse in clients who:

- Are above the age of 45 years
- Smoke
- Use alcohol
- Have pre-existing medical conditions, such as diabetes, high blood pressure, or high cholesterol.

Some clients will experience a reduction in their blood pressure, and improvements in their cholesterol levels on hormone therapy. This is not a guarantee and is not a replacement for positive lifestyle changes.

Monitoring and follow up

Your doctor will advise and guide you in monitoring your safety while you are on hormone therapy. Usually, this will involve regular check-ups and physical examinations, as well as certain blood tests.

At the outset, it is not uncommon for these evaluations to be done monthly, while you are still achieving the correct hormonal balance for you. Later, once you are stable on treatment, these intervals might be extended to six-monthly, or perhaps even annually. This schedule is different for every client.

Stopping hormone therapy

If you decide to stop your hormone therapy, **you should discuss this decision with your doctor.** It can be dangerous to abruptly withdraw hormone therapy without adequate medical supervision.

Please contact the practice through our whatsapp line +2774 893 3296 or email: infomydr@gmail.com with any questions or concerns while on treatment.

Please note that the turn-around time for email queries can be up to 48 hours and longer on weekends or public holidays.

IMPORTANTLY, for anything urgent please call or send a whatsapp message to the practice using +2774 893 3296 or use your nearest hospital emergency facility if after hours.

More information

1. Information on self-injection technique can be found at:

Youtube:

- 1) How to perform a subcutaneous injection - gender affirming hormone therapy - Dr Anastacia Tomson

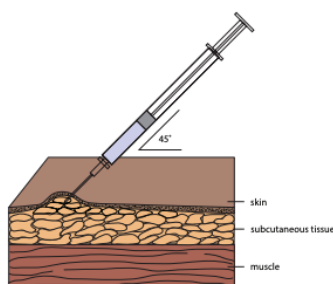
<https://www.youtube.com/watch?v=1uehXqNh8p4>

WHAT IS A SUBCUTANEOUS INJECTION?

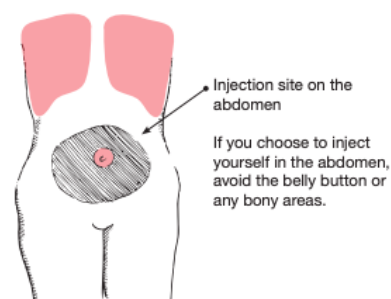
A subcutaneous injection, delivers medication into the subcutaneous layer of tissue that is directly under the skin.

Medication delivered this way is more slowly absorbed into the bloodstream than medication that is directly injected into the muscle.

Needle for SubQ injections are usually 23-25 Gauge and 5/8" (inch) in length. Medication should never be drawn up through these needles. Use a separate drawing up needle.



Whichever site you choose to inject, you should be able to firmly grasp and pinch up approximately a 1-inch fold of skin.



2. Costs of hormone therapy

It is important to remember that not only do different clients have different needs in accessing hormone therapy, but also that prices for medication may vary between different pharmacies, and that these prices may fluctuate over time. The majority of clients can expect to spend approximately R600–R800 per month on hormone therapy. This does not include the cost of monitoring blood tests or doctor's visits.

Feminising Hormone therapy Cost Guide (edited. Aug 2026)		
Initial Consultation with baseline bloods		
	In-person consultation (30 mins)	Video consultation (30 mins)
Initial Treatment Assessment Consultation	R600 (including in-room HIV & syphilis testing)	R500
Baseline blood tests Hb; Hepatitis; Creatinine; ALT; T.Chol (+ Glucose/Syphilis/HIV if virtual consult)	+/-R265 (invoiced by Lab.)*	R410 (invoiced by Lab.)*
Video Consultation to review blood results & dosage	R335	

Medication options	Estradiol Valerate (Fagron)	R589.00 every 3 months (+ R125-R200 delivery fee)
	Estrofem + Spironolactone	+/- R 1200 Per Month (Dischem 23/01/2026)
	Premarin + Spironolactone	+/- R1100 Per Month (Dischem 23/01/2026)

***Laboratory testing is not included in the doctor's fee. We provide you with a pathology request form and you'll pay the laboratory directly.**

Follow up Blood tests and consultation	
Monitoring blood tests (Hamadi Labs) Hb; Creat; ALT; Total Estrogen; Testosterone	+/-R265 (invoiced by Lab.)*
Video Consultation to review blood results & dosage	R335

*Pricing subject to change

3. Blood testing and Follow up consultations

a) Initial Blood test – New clients

1. Complete your Initial Treatment Assessment Consultation

During your initial treatment assessment consultation, your doctor will assess your health, discuss your gender-affirming treatment goals and determine which blood tests are required before starting treatment.

For patients consulting in-person at our rooms (65a Dennegeur Avenue, Strandfontein, Cape Town), we offer on-site specimen collection.

For patients consulting via video consultation, you will receive a pathology request form to have your blood tests done at our recommended laboratory.

2. Have your blood tests done

We can recommend a pathology laboratory that offers competitive cash-paying rates. Please let us know if you would prefer to use a different laboratory that is more convenient for you.

Your **Initial** blood tests can be done:

- Any day of the week
- At any time during practice and laboratory office hours
- You do not need to wait for a specific day or time unless your doctor has given you specific instructions

Please take your pathology request form with you when you have your blood drawn at the lab.

3. Book your results consultation

Once your blood test has been completed, you can book a video consultation for approximately 2 days after your blood test.

This gives the laboratory time to process your results and allows your doctor to review them before your consultation.

During this video consultation, your doctor will:

- Review your blood test results with you
- Explain what they mean
- Confirm whether it is appropriate to proceed with treatment
- Discuss your treatment options and next steps
- Explain your medication and dosing where appropriate
- Discuss when your next blood tests will be required

b) Follow up blood test – New and existing clients

1. Your follow-up blood tests

Your 1st follow-up blood tests will be done **approximately 1 month after starting your medication** to check your hormone levels and make sure that your dose is appropriate.

If you are using a **weekly injection**, your blood test must be done **on the day of your 5th-week injection, BEFORE you give yourself that week's dose.**

This timing is extremely important. Your hormone levels change depending on where you are in your injection cycle. Having the blood test at the wrong time may make the results difficult to interpret and could mean that the blood test needs to be repeated, resulting in an unnecessary additional cost.

For patients taking oral hormones this blood test can be done during your 5th week on treatment:

- Any day of that week
- At any time during practice and laboratory office hours
- You do not need to wait for a specific day or time unless your doctor has given you specific instructions

2. Book your results consultation

Once your blood test has been completed, you can book a video consultation for approximately 2 days after your blood test.

This gives the laboratory time to process your results and allows your doctor to review them before your consultation.

During this video consultation, your doctor will:

- Review your blood test results with you
- Explain what the results mean
- Discuss any effects or side-effects you have experienced since starting hormone treatment.
- Adjust medication dosage where appropriate
- Discuss when your next blood tests will be required

4. How to order your injectable medication

Estadiol Valerate can only be ordered through a compounding pharmacy like Fagron.

- Go to the Fagron online ordering website and create a new patient account:
Fagron – New Patient Order
(<https://fagron.co.za/place-an-order/place-an-order-new-patients/>)
- Upload ONLY the prescription for your Estradiol Valerate vial when completing your Fagron order.
- Do not upload the prescription containing your syringes and needles to Fagron.
- Your separate prescription for syringes and needles can be taken or sent to your nearest pharmacy for dispensing and collection.
- If you have any difficulty placing your order, please contact the practice for assistance.

5. Home affairs and gender marker (South African Nationals ONLY)

If you are a South African Citizen and wish to update your gender marker on your birth certificate and identity document (ID), then the Department of Home Affairs requires two letters from healthcare professionals which state that you have undergone medical or surgical gender reassignment. Either of these is sufficient; you do not need to have had surgery to update your gender marker.

Unfortunately, at present, gender markers in South Africa are binary; thus, an ID document can reflect either female or male. There is no unspecified or non-binary marker.

Your prescribing doctor can write one of your letters for the Department of Home Affairs and should be able to refer you to another healthcare provider to write the second letter.

If you wish to change your name, then this process should be undertaken separately (either before or after) to the process of updating your gender marker. This does not require supporting letters from healthcare providers.

Potential challenges with legal documents: Some TGD persons have trouble with banking, registration as a student and writing examinations, registration of motor vehicles, etc. while they are awaiting their new documents. Some are accused of potential fraud because they do not look the same as the photo in their ID or their name may be different from the name on their qualification certificates. To change one's gender marker and names will take a while. You may want to consider how you will deal with these challenges in the interim. You may also want to consider if you need to change other documentation, for example your matric certificate.

6. Fertility

Although not all clients become infertile on hormone therapy, and some might regain fertility if they stop hormone therapy, many may become irreversibly infertile. Hormone therapy is not a replacement for effective and responsible contraceptive use.

All clients considering starting on hormone therapy should consider using a cryobank to preserve genetic material, in case they wish to conceive genetically related children at a later stage. Even if this is not a priority for you at this stage in your life, please consider the possibility that your perspectives might change with time, and that it is ideal to store material before starting hormone therapy, rather than trying to regain fertility once you are already taking hormones.

Fertility Preservation for Trans Women (Sperm freezing / sperm cryopreservation) [in Cape Town, South Africa]

Sperm Freezing (Cryopreservation) –Allows you to preserve the option of having biological children in the future.

Sperm freezing (also called cryopreservation) involves collecting, freezing, and safely storing sperm for future use. Stored sperm can be kept for many years.

Oestrogen and testosterone blockers can reduce or stop sperm production, sometimes permanently. Freezing sperm before starting hormones keeps future family-building options open. You do not need to know yet whether you want children.

Trans women and transfeminine people or anyone producing sperm who wants to preserve fertility before hormone therapy or surgery can use this service.

The process involves one or two clinic visits; Private sperm sample collection; Testing, freezing, and secure storage; Annual storage with the option to continue long-term. The process is safe, confidential, and respectful.

Sperm freezing is offered through fertility clinics in Cape Town, including:

Cape Fertility

HART Fertility Clinic

Aevitas Fertility Clinic

Feel free to contact any of these facilities for more information related to their specific service.

Costs (approximate – Cape Town, South Africa at time this document was edited)

Prices vary by clinic

Initial freezing + first year storage: ± R3,500 – R7,000

Annual storage thereafter: Clinic-specific (you will be informed upfront)

Important to know

Freezing sperm does not significantly delay starting hormone therapy
Hormones do not reliably restore fertility once sperm production stops
Stored sperm can later be used with IVF or other fertility treatments
This is a personal choice, there is no pressure to proceed.

Disclaimer

This document has been developed by My Doctor Family Practice as a practical resource to support clients accessing gender-affirming healthcare. It incorporates and summarises selected recommendations from established clinical guidance, including the Southern African HIV Clinicians Society (SAHCS) Gender-Affirming Healthcare Guideline for South Africa (2021), together with additional information based on clinical experience, current practice, and the processes followed by My Doctor Family Practice.

This document is intended to provide general educational and practical information and should not be regarded as a substitute for the full published clinical guidelines, professional training, or individual clinical assessment and judgement. Clinical recommendations and treatment decisions must be tailored to each patient's individual circumstances, medical history, goals, risks, preferences and response to treatment.

The information in this guide reflects the evidence, clinical practice, medication availability, laboratory services and practice procedures available at the time of publication. Gender-affirming healthcare is an evolving field, and recommendations, treatment options, monitoring requirements, medication availability and costs may change as new evidence and guidance become available. The most current relevant clinical guidelines and prescribing information should therefore always be consulted.

Information relating specifically to My Doctor Family Practice, including consultation processes, estimated costs, laboratory testing arrangements, medication-ordering procedures and other practical instructions, is provided for the convenience of our patients and reflects our practice's current procedures. These details are subject to change and should not be interpreted as universal requirements or recommendations applicable to all healthcare providers or practices. Costs provided are estimates only and may vary depending on the laboratory, pharmacy, medication supplier, medical aid benefits and other external factors.

Patients should not use this document as a basis for independently starting, stopping or changing hormone therapy or other medication. Medication should be taken only as prescribed by an appropriately qualified healthcare professional, and any concerns regarding treatment should be discussed with the treating clinician.

Where this document differs from a current national or international clinical guideline, the **most current authoritative guidance and the individual patient's clinical circumstances should take precedence.**

This document is intended to complement, rather than replace, personalised medical care.

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